

---

# Extending the reach of specialist paediatric expertise *across* *Queensland*

Strategic options and recommendation for Children's Health Queensland  
(CHQ)

DANIELA · JACINDA · LINDA · RASHMI

How can CHQ extend the *reach* of its specialist expertise across Queensland while protecting the *concentration* of capability that quaternary excellence needs?

# Three gaps define CHQ's *strategic challenge*

## 01 • CAPABILITY GAP

37%

of Category 2 outpatients are seen on time.

Demand growing 2.9–5.1% a year against 1.2% population growth. Vacancy rate 14.2%; staff engagement 68%.

## 02 • GEOGRAPHIC GAP

80%

of Greater Brisbane's extra children will live outside QCH's catchment.

125,953 of 156,990. Growth corridors such as Ipswich (+3.2% a year) sit far from the specialist hub.

## 03 • OPERATING-MODEL GAP

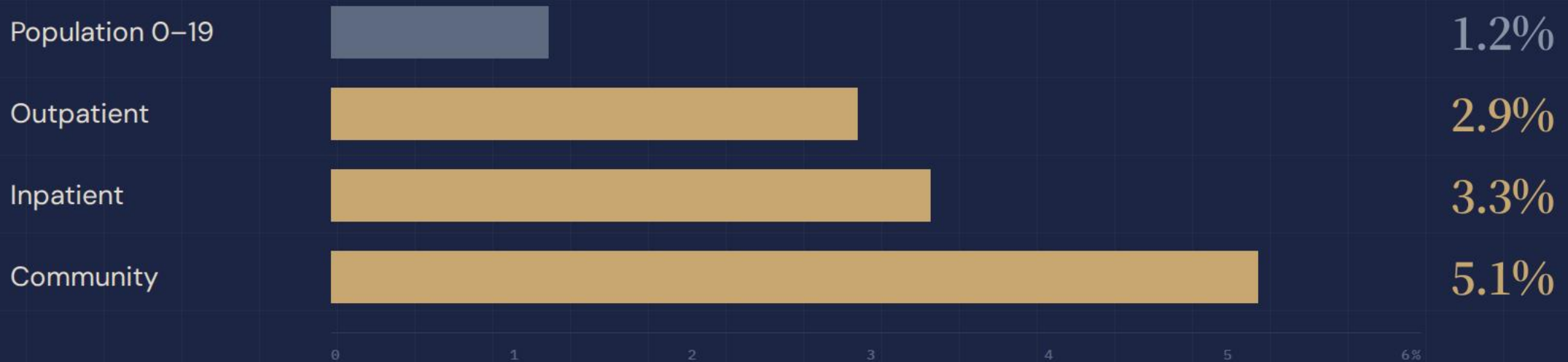
0 of 7

networked-care KPIs reported in 2024–25.

The statewide role is not mandated, funded or governed. Operating surplus fell from \$10.99m to \$0.13m.

# Demand is growing *two to four times* faster than the children CHQ serves

PROJECTED GROWTH, % A YEAR (MASTER PLAN)



# Scarce specialist expertise is both CHQ's *advantage* and its *constraint*

## PESTEL

Policy and demographics push care *closer to home*.

## Five Forces

Supplier power high (scarce specialists); buyer power high (CHQ cannot direct HHSs); rivalry and entrants low.

## VRIN

Quaternary expertise is a sustained advantage, but the same people carry care, training and statewide support.

## Budget

Expenses +8.5% vs income +7.4% in 2024–25.

THE INSIGHT WE CARRY FORWARD

---

Centralisation creates capability *depth*.  
Networking creates capability *reach*.

# Three options, each a *different degree* of change

## OPTION 1

Extension of the existing path

### Extend networked care

Scale advice, outreach, shared care and local follow-up in 2–3 pathways with willing HHS partners.

RISK • stays fragmented if gains are not codified.

## OPTION 2

Radical change in the existing market

### Network orchestrator

CHQ and HHSs jointly set statewide pathways, standards, shared records and a dashboard.

RISK • accountability without authority or funding.

## OPTION 3

Expansion into new segments

### New markets

Sell education, research partnerships and selected IP beyond Queensland.

RISK • diverts scarce clinicians; returns uncertain.

UNIT OF CHANGE • specific pathways (1) › the whole network's operating model (2) › new external customers (3)

# Option 1 is the only option with *no weak test*

| CRITERION (WEIGHT)              | OPTION 1                                                                              | OPTION 2                                                                              | OPTION 3                                                                              |
|---------------------------------|---------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------|---------------------------------------------------------------------------------------|
| Suitability 30%                 | 4  | 5  | 2  |
| Feasibility 25%                 | 4  | 2  | 3  |
| Acceptability: stakeholders 20% | 4  | 3  | 3  |
| Acceptability: risk 15%         | 4  | 2  | 3  |
| Value / return 10%              | 3  | 4  | 2  |
| Weighted total                  | 3.9                                                                                   | 3.3                                                                                   | 2.6                                                                                   |

ROBUSTNESS • 10,000 RANDOM WEIGHTINGS

# 97.8%

of weightings rank Option 1 first.

Option 2 wins only if its feasibility reaches *5 out of 5*.

That needs a mandate, shared data and funding — precisely what an Option 1 pilot would help secure.

OPTION 1 • 97.8%

OPTION 2 • 2.2% • OPTION 3 • 0%

Option 2 is the best *long-term* fit; Option 1 is the best *deliverable* fit — and the route to Option 2.

# Recommendation: an *ECHO-led, stage-gated* Option 1

Approve Option 1 as CHQ's preferred strategy for 2026–2028: an 18-month, jointly governed pathway program in three pathways.

01 Type 1 diabetes

CENTRAL QUEENSLAND

02 Cleft care

TOWNSVILLE

03 General paediatrics and neurology

WEST MORETON

## How — and why — we *modified* Option 1

---

1

ECHO-led workforce redesign at the core

Targets the capability gap.

---

2

Joint governance with partner HHSs

Targets the operating-model gap.

---

3

Funded data foundations and AI assurance

\$0.45m workstream.

---

4

Stage gates and stop rules

Option 2 decided at month 18; Option 3 deferred.

Value now, with the *option to transform later*: Option 1 is the practical first step to Option 2.

Better access and a protected specialist workforce, for a *small, capped* investment

\$4.4<sub>m</sub>

18-month program  
0.38% of annual spend

~\$95<sub>m</sub>

QCH capital avoided  
in a working network

73.7

fewer additional QCH beds  
by 2036/37

## Value for *every* key stakeholder

|                       |                                                   |
|-----------------------|---------------------------------------------------|
| Children and families | Less travel, faster follow-up, named coordinator. |
|-----------------------|---------------------------------------------------|

|                |                                                             |
|----------------|-------------------------------------------------------------|
| CHQ clinicians | Specialist time on complex care; teaching and career roles. |
|----------------|-------------------------------------------------------------|

|              |                                         |
|--------------|-----------------------------------------|
| Partner HHSs | Local capability; ABF retained locally. |
|--------------|-----------------------------------------|

|                      |                                              |
|----------------------|----------------------------------------------|
| Department of Health | Service Agreement evidence; capital avoided. |
|----------------------|----------------------------------------------|

# 18 months, four phases, *Board gates* at every step

MONTHS 0-3

## Mobilise

Governance group,  
MOUs, baselines

MONTHS 3-6

## Co-design

Pathway agreements,  
safety thresholds

MONTHS 6-15

## Pilot

Waves, ECHO, dashboard

MONTHS 15-18

## Evaluate and decide

Scale, adapt or stop

GATE • MONTH 6

GATE • MONTH 12

GATE • MONTH 18

## What success looks like at *month 18*

Eligible follow-up completed locally

$\geq 50\%$

Median wait, pilot specialties

65 days →  $-10\%$

Networked-care KPIs reported

0 of 7 → **7 of 7**

Unplanned escalation and adverse events

*No sustained increase*

## Four *decisions* we ask of the Board

---

1

Endorse Option 1 for 2026–2028

---

2

Approve \$4.4m, 18-month program with gates at 6, 12, 18 months

---

3

Seek Service Agreement recognition of the statewide role

---

4

Defer Option 2 to a month–18 business case; Option 3 to a six-month asset review

# The main risks are *manageable*

|                           |                                                                |
|---------------------------|----------------------------------------------------------------|
| Clinical safety           | CSCF eligibility, credentialing, CATCH escalation, stop rules. |
| Specialist workload rises | Fund protected sessions first.                                 |
| Partner capacity          | Backfill, right to decline.                                    |
| Data gaps                 | Workstream 8 and data-readiness gate.                          |

---

PROTECT

Keep complex, high-acuity and genuinely quaternary care *concentrated at QCH*.

---

EXTEND

Use networked care, ECHO®, shared-care pathways and local capability to bring appropriate care *closer to families*.

---

BUILD

Use the 18-month stage-gated pilot to establish evidence, governance and data foundations for a *future network-orchestrator* model.

Option 1 delivers value now — while building CHQ's capability to transform later.

OUR ANSWER TO THE OPENING QUESTION

---

Extend reach by moving *capability*, not  
by moving the specialists.

QUESTIONS & DISCUSSION

FOR QUESTIONS

---

# Appendices: method, evidence and *implementation detail*



## Research method and *evidence base*

- **Primary sources.** CHQ Annual Report 2024–25, Master Plan 2021/22–2036/37, Strategic Plan 2024–2028, Services Plan 2018–2028, Digital and Data Strategy 2024–2028, Operational Plan FY2025–26
- **Assessment 1 frameworks.** PESTEL, Five Forces and strategic groups, VRIN/VRIO, value chain; CHQ consultation (9 Sep 2026)
- **External evidence.** Peer-reviewed evaluations of paediatric telehealth and Project ECHO; SickKids and GOSH benchmarks
- **Analysis methods.** Weighted SAF scoring; Monte Carlo sensitivity (10,000 Dirichlet draws); Master Plan capacity scenarios; Mendelow stakeholder mapping; risk register
- **Limits.** Scores are team judgement; budget indicative; some studies read from abstracts; refresh KPIs with the 2025–26 Annual Report

# Options and core *analytical insights*

|                  | OPTION 1                                                             | OPTION 2                                                           | OPTION 3                                                        |
|------------------|----------------------------------------------------------------------|--------------------------------------------------------------------|-----------------------------------------------------------------|
| Core idea        | Scale advice, outreach, shared care, local follow-up in 2–3 pathways | CHQ and HHSs jointly govern statewide pathways, records, dashboard | Package education, research, IP for external buyers             |
| Builds on        | Specialist Online Advice, CATCH, ECHO, Rockhampton and Townsville    | Master Plan fully networked scenario; audiology governance model   | Paid masterclasses; 805 publications; \$24.16m research funding |
| Key insight used | Specialist time is the binding constraint; telehealth 125% of target | Root cause is absence of governed statewide system                 | Expertise is rare and hard to imitate (VRIN)                    |
| Data need        | Pilot-scale (gap 0.9 pts)                                            | Statewide, near-real-time (gap 2.4 pts)                            | De-identified data and IP rules first                           |

## Detailed evaluation and *decision criteria*

| CRITERION                   | WEIGHT     | QUESTION AND RATIONALE                                                                               |
|-----------------------------|------------|------------------------------------------------------------------------------------------------------|
| Suitability                 | <b>30%</b> | Does it fix the specialist bottleneck, growth outside the catchment and the unmeasured network role? |
| Feasibility                 | <b>25%</b> | Deliverable in 1–3 years given 14.2% vacancy, near-zero surplus, legacy digital                      |
| Acceptability: stakeholders | <b>20%</b> | Families, clinicians, HHSs, Department; CHQ cannot direct HHSs                                       |
| Acceptability: risk         | <b>15%</b> | Clinical, financial, reputational, governance exposure                                               |
| Value / return              | <b>10%</b> | Long-run access, capacity and cost benefit; uncertain for all                                        |

| SCENARIO                       | OPT 1              | OPT 2       | OPT 3 |
|--------------------------------|--------------------|-------------|-------|
| Base case                      | <b>3.90</b>        | 3.30        | 2.60  |
| Suitability-led 45/15/15/10/15 | <b>3.85</b>        | 3.80        | 2.40  |
| Equal weights                  | <b>3.80</b>        | 3.20        | 2.60  |
| Opt 2 feasibility = 4          | <b>3.90</b>        | 3.80        | 2.60  |
| Opt 2 feasibility = 5          | 3.90               | <b>4.05</b> | 2.60  |
| 10,000 random weights          | <b>97.8% first</b> | 2.2% first  | 0%    |

## Selected option — *modifications* and rationale

| MODIFICATION                                              | WHY                                                          | BENEFIT TO CHQ AND STAKEHOLDERS                                                             |
|-----------------------------------------------------------|--------------------------------------------------------------|---------------------------------------------------------------------------------------------|
| ECHO-led workforce redesign at the core                   | Specialist workforce is the binding constraint               | Specialists focus on quaternary care; GPs, nurses and allied health gain skills and careers |
| Joint Pathway Governance Group                            | CHQ cannot direct other HHSs                                 | Shared accountability; partner buy-in                                                       |
| Workstream 8: data foundations and AI assurance (\$0.45m) | 0 of 7 networked-care KPIs reported; data maturity 1.9/5     | Measurable network; safe AI; gate for Option 2                                              |
| Capped budget, stage gates, stop rules                    | \$0.13m surplus; no financial headroom                       | Reversible; limits downside; tests \$95m capital case                                       |
| Option 2 deferred to month 18; Option 3 to asset review   | Option 2 lacks mandate and data; Option 3 diverts clinicians | Preserves the option to transform                                                           |

## Evidence for the *ECHO-led tiered* workforce model

| ROLE                                                       | FUNCTION                                           | QUEENSLAND EVIDENCE                                                          |
|------------------------------------------------------------|----------------------------------------------------|------------------------------------------------------------------------------|
| GPs and primary care                                       | Present before referring; manage stable conditions | ADHD ECHO: self-efficacy 2.6 to 5.4 of 7; SC4C trial: referrals 7.3% to 3.0% |
| Nurse practitioners                                        | Protocol-based triage; chronic condition clinics   | 9 CHQ subspecialties; 85% of clinicians rate a success                       |
| Nurse navigators                                           | Named case manager across services                 | 400-navigator program, gains on all hospital measures                        |
| Advanced-practice allied health                            | First assessment of Cat 2–3 referrals              | Sunshine Coast MPSC-MDT: about 96% managed without surgeon                   |
| Aboriginal and Torres Strait Islander health practitioners | Community screening and follow-up                  | Deadly Ears: 563 assessments beside 556 ENT-seen children                    |
| GP-led clinic in QCH ED                                    | Low-acuity presentations                           | About 25% of presentations; ED waits cut 21.6 min                            |

# Benefits to CHQ and *key stakeholders*

| STAKEHOLDER                                    | RISK OR CONCERN                                         | RESPONSE BUILT INTO THE PLAN                                         |
|------------------------------------------------|---------------------------------------------------------|----------------------------------------------------------------------|
| Children and families                          | Local care seen as a downgrade; delayed escalation      | Co-designed consent; family choice; guaranteed rapid CHQ review      |
| CHQ clinicians                                 | Advice on top of full clinics; burnout (68% engagement) | Protected, funded sessions; clinicians co-design pathways            |
| Partner HHSs                                   | Unfunded clinics; liability; recruitment                | Signed agreements covering funding, CSCF scope, right to decline     |
| Department of Health                           | Precedent for a new funding line                        | Pilot as evidence base for Service Agreement role                    |
| Aboriginal and Torres Strait Islander families | Culturally unsafe care; widening gaps                   | Involve IUIH and community-controlled services; report by population |

# Detailed *implementation* plan

- **Pilot pathways.** Type 1 diabetes and insulin pumps (Central Queensland); cleft lip and palate follow-up (Townsville); general paediatric and neurology follow-up (West Moreton, Ipswich +3.2% a year)
- **Eight workstreams.** Governance; referral front door; advice and virtual care; shared care; workforce redesign and ECHO; coordination, families and equity; data and evaluation; data foundations and AI assurance
- **Networked Care Office.** A small office in Strategy and Transformation becomes the nucleus of any later orchestrator role
- **Change approach.** Kotter's sequence; early wins from Rockhampton and Townsville

| BUDGET LINE                         | \$M         |
|-------------------------------------|-------------|
| Protected specialist advice time    | 1.05        |
| Advanced-practice roles (3.0 FTE)   | 0.68        |
| Nurse navigators (3.0 FTE)          | 0.60        |
| Partner HHS backfill and training   | 0.55        |
| Network data foundations and AI     | 0.45        |
| Program management and analyst      | 0.40        |
| ECHO 2.0 expansion                  | 0.35        |
| Telehealth and record configuration | 0.15        |
| Independent evaluation              | 0.15        |
| <b>Total</b>                        | <b>4.38</b> |

# Risks, funding safeguards and *data-readiness gate*

| RISK                               | RATING        | MITIGATION                                                                        |
|------------------------------------|---------------|-----------------------------------------------------------------------------------|
| Care moves beyond local capability | Medium / High | CSCF eligibility, credentialing, CATCH 24/7, stop rules                           |
| Specialist workload rises          | High / High   | Fund protected sessions first; track minutes per patient                          |
| Loss of ABF activity               | High / Medium | Telehealth review keeps CHQ revenue; refill freed slots; negotiate advice funding |
| Data cannot be shared or linked    | Medium / High | Data-sharing agreements, privacy impact assessments, The Viewer                   |
| Unsafe or biased AI                | Low / High    | Silent mode first; clinician decides; AI register                                 |

OPTION 2 GATE · network data set live in 3+ HHSs at 95%+ audited accuracy; all 7 networked-care KPIs reported; data-sharing agreements signed; shared care plan in routine use; platform funded; AI tools risk-assessed.

# Generative AI *acknowledgement*

## AI TOOL AND NATURE OF USE

- **Tool.** OpenAI ChatGPT — GPT-5.6 Sol.
- **How it was used.** To support synthesis of the team's CHQ research, presentation structure, concise slide wording, speaker-note drafting and visual organisation.
- **Scope.** Main presentation slides and appendices. AI was used as an assistive drafting and synthesis tool; it was not treated as an authoritative source of CHQ facts.
- **Date.** September 2026.